

Crisis Intervention Behavioral Health Level of Care

ORG: B-905-CI (BHG)

MCG Health
Behavioral Health
Care
28th Edition

This MCG guideline has not yet been updated to reflect new guidance and requirements found in the 2024 Medicare Advantage and Part D Final Rule (CMS-4201-F). For Medicare Advantage beneficiaries, before referring to MCG's guidelines users should first follow all guidance related to this topic located in CMS National Coverage Determinations (NCDs), Local Coverage Determinations (LCDs), and other statutes and regulations. Content used in MCG guidelines may be used to supplement but does not replace, modify, or supersede existing Medicare regulations or applicable NCDs or LCDs.

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Admission Guidelines

- Admission to **Crisis Intervention Level of Care** (LOCUS/CALOCUS-CASII Basic Services) is indicated due to **ALL** of the following^{[A][B]}(3)(15)(16)(17)(18)(19)(20):
 - Around-the-clock behavioral care or close monitoring and intervention is indicated by **1 or more** of the following⁽²¹⁾⁽²²⁾⁽²³⁾:
 - Danger to self for adult or Danger to self for child or adolescent
 - Danger to others for adult or Danger to others for child or adolescent
 - **Serious** dysfunction in daily living for adult or **Serious** dysfunction in daily living for child or adolescent
 - ☐ Patient management/treatment at lower level of care is not feasible (eg, lower level of care is unavailable or is not appropriate for patient condition) until acute intervention or modification (eg, acute medication, identification of needed resources and support, change in living situation) is initiated.^[C]
 - ☐ Situation and expectations are appropriate for crisis intervention services, as indicated by **ALL** of the following⁽⁶⁾⁽⁸⁾⁽²⁴⁾⁽²⁵⁾:
 - Recommended treatment is necessary, appropriate, and not feasible at lower level of care (eg, lower level of care is inappropriate for patient condition or is not available).^[D]
 - Sufficient treatment outcome (eg, stabilization and identification of resources and support for care outside of crisis intervention services) is expected within short time period (eg, 1 to 3 days).
 - Patient is willing to participate in treatment (or agrees to participate at direction of parent or guardian) within specified intervention and treatment structure voluntarily (or due to court order).^{[E][F]}
 - There is no anticipated need for physical restraint, seclusion, or other involuntary control (eg, patient not actively violent).
 - There is no need for around-the-clock medical or nursing care.
 - Patient has sufficient ability to respond as planned to individual and group therapeutic interventions.^[G]
 - Biopsychosocial stressors^[H] potentially contributing to clinical presentation (eg, comorbidities,^{[I][J]} illness history, environment,^[K] social network, ability to cope, and level of engagement^[L]) have been assessed and are absent or manageable at proposed level of care.⁽⁶⁾⁽⁷⁾⁽⁸⁾⁽⁹⁾⁽¹⁰⁾⁽¹¹⁾⁽¹²⁾⁽¹⁴⁾⁽³³⁾⁽³⁴⁾

Recovery Course

Stage	Clinical Status	Interventions	Evaluation
1	• Clinical Indications met^[M] • Destabilizing events or situation identified with appropriate interventions and treatment instituted	• Appropriate crisis management instituted • Begin Care coordination • Regular safety checks done at appropriate frequency	• Evaluation completed and reviewed • Symptom and functioning assessment at appropriate frequency documented

	• Social Determinants of Health Assessment • Begin Discharge planning		• Physician evaluation of patient and progress performed or documented as not needed • Review of need for medication or adjustment at appropriate frequency documented
2	• Risk status acceptable • Functional status acceptable • Social Determinants of Health Assessment • Complete Discharge planning • Discharge	• Medical needs absent or manageable/treatable at available lower level of care • Treatment plan for follow-up care and safety monitoring (eg, safety plan) completed[N]	

(8)(20)(21)(23)(36)(37)

Recovery Milestones are indicated in **bold**.

Discharge Guidelines

- Continued crisis care is generally needed until **1 or more** of the following:
 - Crisis care is no longer necessary due to adequate patient stabilization or improvement, as indicated by **ALL** of the following(20)(21)(23):
 - Risk status acceptable, as indicated by **ALL** of the following(6)(7)(10)(12)(24)(38):
 - Danger to self or others manageable/treatable, as indicated by **1 or more** of the following:
 - Absence of Thoughts of suicide, homicide, or serious Harm to self or to another
 - Thoughts of suicide, homicide, or serious Harm to self or to another present but manageable/treatable at available lower level of care
 - Patient and supports understand follow-up treatment and crisis plan.
 - Provider and supports are sufficiently available at lower level of care.
 - Patient, as appropriate, can participate as needed in monitoring at available lower level of care.
 - Functional status acceptable, as indicated by **1 or more** of the following:
 - No essential function[O] is significantly impaired.
 - An essential function is impaired, but impairment is manageable/treatable at available lower level of care.
 - Medical needs absent or manageable/treatable at available lower level of care, as indicated by **ALL** of the following(6)(7)(10)(12)(24)(38):
 - Adverse medication effects absent or manageable/treatable
 - Medical comorbidity absent or manageable/treatable
 - Medical complications absent or manageable/treatable (eg, complications of eating disorder)
 - Substance-related disorder absent or manageable/treatable
 - Treatment plan for follow-up care and safety monitoring (eg, safety plan) completed[N]
 - Crisis care is no longer indicated due to **1 or more** of the following:
 - Higher level of care is indicated (eg, patient condition has deteriorated, greater service intensity is necessary to support engagement in care or reinforce skills, or more intensive supervision is necessary to address clinical needs).
 - Patient or guardian refuses treatment.[P]

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Footnotes

[A] Short-term (1 to 3 days) crisis stabilization can be accomplished by crisis intervention care that can be done in a facility setting (eg, residential care, short-term psychiatric unit) or by outreach teams.(1)(2)(3) For voluntary patients with acute manifestations of psychiatric disorders who are being considered for inpatient admission because of clinical urgency but are not expected to require physical restraint or around-the-clock nursing care, randomized controlled trials have shown that a residential setting can provide treatment that is no less effective than inpatient care.(1)(4)(5) [A in Context Link 1]

[B] The purpose of the care guidelines is to promote evidence-based care across the continuum of care to enhance the delivery of quality healthcare. Indications are presented for different levels of care. These indications help define the optimal level of care and can assist in developing alternatives to higher levels of care, tracking patient progress during treatment within a level of care, facilitating the progress of patients whose recovery is delayed, and preparing comprehensive plans for transition of patients from one level of care to another. Relevant professional society guidelines are foundational content for evaluation and treatment of behavioral health disorders at different levels of care and are complemented by the best available published evidence.(6)(7)(8)(9)(10)(11)(12)(13)(14) [B in Context Link 1]

[C] Acute interventions may address multiple factors contributing to a patient's condition including medication needs, identification of needed resources and support, or change in living situation.(17) [C in Context Link 1]

[D] Goals for care may be diagnostic, therapeutic, or rehabilitative in nature. Goals should be agreed upon by the patient (or guardian, as appropriate), and incorporate the patient's perspective on current problems, as well as the patient's specific values and preferences. Lack of progress toward goals should trigger a reassessment of the care plan (eg, determination as to accuracy of the admitting diagnosis, identification of barriers toward progress, incorporation of new problems that may have developed into the treatment plan, assessment of the appropriateness of initial management strategies), and lead to the modification of the plan as appropriate to promote optimal recovery.(14)(26) [D in Context Link 1]

[E] Evaluation of the lack of willingness to participate in treatment (eg, refusal to consent to or participate in treatment) should ascertain whether or not additional efforts to engage the patient in treatment are likely to be successful. It should also include a risk assessment (eg, whether or not the patient would represent a risk to self or others), as well as determine whether or not pursuing involuntary treatment is indicated.(6)(7)(10)(11) [E in Context Link 1]

[F] Impairments in motivation to participate in treatment, limitations to engagement in care, and resistance to change are common in patients with mental health and substance use disorders, and may represent a feature of the disease process. A lack of motivation may indicate the need for more intensive services in order to help promote recovery or behavior change. Deficits in motivation or resistance to change should be addressed with therapies designed to enhance motivation to participate in treatment and work toward recovery (eg, motivational enhancement therapy). The level of motivation and goals/strategies designed to address motivation should be included in the care plan, and a lack of progress toward goals should trigger a reassessment of the care plan (eg, identification of barriers toward progress, incorporation of new problems which may have developed into the treatment plan, assessment of the appropriateness of initial management strategies), and lead to modification of the plan as appropriate to promote optimal recovery.(26) (27) [F in Context Link 1]

[G] A variety of factors may impact the ability of the patient to respond to treatment, including language, cognitive, and intellectual capacity, as well as the presence of any developmental conditions. Assessment of the ability to respond to planned interventions may help identify subgroups of patients more likely to respond to particular treatments, and help optimize (or modify) the care plan to increase the likelihood of successful recovery. It may also help determine if a specialized treatment setting (eg, dual-diagnosis program) or alternative level of care would be more appropriate to address the patient's needs. Patients who do not initially respond to scheduled therapeutic interventions may respond to modifications of the treatment plans or strategies designed to enhance ability. These include

the addition or substitution of a new medication or psychosocial therapy, adjustment of the medication dose, increase in service intensity (some patients require higher levels of service intensity to provide reinforcement to enhance motivation and promote change), or decrease in service intensity (some patients may find higher levels of service intensity so restrictive as to be burdensome and hinder recovery).(6)(7)(10)(11)(24)(26)(27) [G in Context Link 1]

[H] Biopsychosocial stressors may impact the level of care necessary to manage a psychiatric or behavioral condition, including the ability of the program to meet comprehensive patient needs, ensure treatment adherence, enhance motivation, or prevent relapse (ie, comorbidities, environmental factors, or other barriers may prevent effective treatment at a less intensive level of care than might otherwise be appropriate to the patient's condition). Biopsychosocial assessment factors should be incorporated into care planning, including planned treatment goals, and intensity and duration of interventions. A "whole person care" approach that coordinates care across physical health, behavioral health, and social service sectors may bridge the gap of physical and mental health.(28)(29)(30)(31) Any identified deficits should be manageable by the program directly or through alternative arrangements.(6)(7)(10)(11) [H in Context Link 1]

[I] Comorbid conditions may directly impact the experience of psychiatric symptoms (eg, COPD and anxiety), or may indirectly impact determination of the appropriate venue for care (eg, routine blood sugar and insulin management in people with diabetes). If clinically appropriate, testing related to diagnosis or management (eg, screening for liver and kidney function, hepatitis, HIV, syphilis, tuberculosis) may be performed off-site.(6)(7)(10)(11) Assessment and treatment of co-occurring medical and/or developmental conditions through services and treatment settings capable of rendering integrated care is recommended.(6)(7)(9)(10)(11)(12) A "whole person care" approach that coordinates care across physical health, behavioral health, and social service sectors may bridge the gap of physical and mental health.(28)(29)(30)(31) The level that comorbid medical and/or developmental conditions are present may be described on a continuum (none/absent, low, moderate, and severe) and may impact determination of the appropriate level of care for treatment (ie, admission to a higher level of care).(6)(7)(10)(11)(24) [I in Context Link 1]

[J] Evaluation/assessment and treatment of co-occurring substance use disorders through services and treatment settings capable of rendering integrated care is recommended.(6)(7)(9)(10)(11) A "whole person care" approach that coordinates care across physical health, behavioral health, and social service sectors may bridge the gap of physical and mental health.(28)(29)(30)(31) The level that comorbid substance use disorders contribute to the primary presenting condition may be described on a continuum (none/absent, low, moderate, and severe) and may impact determination of the appropriate level of care for treatment (ie, admission to a higher level of care or specialty dual-diagnosis program).(6)(7)(10)(11)(24) [J in Context Link 1]

[K] The degree of environmental stressors and amount of support in the patient recovery environment should be considered in the context of the clinical presentation in determining the appropriate level of care for treatment. The level of environmental stressors may be low, mild, moderate, high, or extreme. The level of support in the environment may range from absent or minimal to limited to supportive or highly supportive.(6)(7)(9)(10)(11)(12)(24) [K in Context Link 1]

[L] Participation motivated by a wish to avoid negative consequences rather than accept the need to work toward recovery may require more intense monitoring and follow-up.(8)(24)(32) The patient's level of engagement may be described on a continuum: optimal, positive, limited, minimal, or unengaged. Readiness to change may range from actively and willingly engaged to unable to follow treatment recommendations due to clinical condition.(6)(7)(10)(11)(24) [L in Context Link 1]

[M] See Admission Guidelines in this guideline. [M in Context Link 1]

[N] Patients presenting for emergency psychiatric evaluation and treatment may be at significantly elevated risk of suicide in the following year.(35) [N in Context Link 1, 2]

[O] Essential functions are those that are necessary to sustain life, such as feeding and hydrating oneself.(6)(7)(10)(11) [O in Context Link 1]

[P] Management of patient refusal to consent to or receive treatment should include efforts to enhance patient engagement in the treatment (including addressing cultural factors that may be at play), or otherwise modify the care plan to address the patient's clinical needs (eg, continued treatment at the current level or at an alternative care setting appropriate to the clinical condition and agreeable to the patient). It should also determine whether the patient risk (eg, potential harm to self or others) is such that involuntary treatment is indicated.(6)(7)(10)(11) [P in Context Link 1]

Definitions

Care coordination

- Care coordination includes communication with **ALL** of the following(1)(2):
 - Outside psychiatrist, other therapists, and medical care providers as appropriate
 - Patient, family, guardian, and other supports as appropriate
 - All team members in other disciplines involved in ongoing patient care

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Command auditory hallucinations

- An auditory hallucination is a "false perception of sound." This is the most common type of hallucination in psychiatric disorders and typically consists of voices but also may consist of any type of sound. A command auditory hallucination is a false perception of a voice giving orders to the patient that they may feel compelled to obey. Likely adherence to a command auditory hallucination is indicated by **1 or more** of the following(1)(2)(3)(4):
 - Perceived familiar or benevolent source
 - No belief by patient in their ability to control the voices
 - History of frequent response to such hallucinations with unquestioning obedience

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Danger to others for adult

- Danger to others for adult is present due to **1 or more** of the following(1)(2)(3):
 - Command auditory hallucinations or paranoid delusions contributing to risk for homicide or serious Harm to another
 - Patient has persistent thoughts of homicide or serious Harm to another that cannot be adequately monitored or treated at lower level of care, as indicated by **1 or more** of the following[A]:
 - Necessary behavioral health care (such as required provider or lower level of care) not available or insufficient
 - Patient support system inadequate
 - Patient characteristics, such as moderately severe to severely Impaired impulse control, judgment, or insight; moderate to moderately severe Mania (adult) or unreliability
 - Indication or report of significant physical or sexual aggression with Impaired impulse control, judgment, or insight that significantly endangers another
 - Current homicidal ideation with either clearly expressed intentions or past history of carrying out such behavior

References

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Footnotes

- A. Patients with persistent thoughts of suicide, homicide, or serious harm to self or another may need frequent monitoring for progression to planning or intention to act on their thoughts. Such monitoring can be safely conducted at an outpatient level of care if providers and supports are adequately available and the patient can reliably participate in the monitoring process.(2)

Danger to others for child or adolescent

- Danger to others for child or adolescent is present due to **1 or more** of the following(1)(2)(3)(4)(5)(6):

- Command auditory hallucinations or paranoid delusions contributing to risk for homicide or serious Harm to another
- Patient has persistent thoughts of homicide or serious Harm to another that cannot be adequately monitored or treated at lower level of care, as indicated by **1 or more** of the following[A]:
 - Necessary child or adolescent behavioral health care (such as required provider or lower level of care) not available or insufficient
 - Conflict in family environment or other inadequacy in patient support system
 - Patient characteristics, such as moderately severe to severely Impaired impulse control, judgment, or insight; moderate to moderately severe Mania (child or adolescent) or unreliability
 - Indication or report of significant physical or sexual aggression with Impaired impulse control, judgment, or insight that significantly endangers another
 - Current homicidal ideation with either clearly expressed intentions or past history of carrying out such behavior

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7. Sewall CJR, Goldstein TR, Brent DA. Youth suicide. In: Dulcan MK, editor. Dulcan's Textbook of Child and Adolescent Psychiatry. 3rd ed. American Psychiatric Association Publishing; 2022:551-564.

Footnotes

- A. Patients with persistent thoughts of suicide, homicide, or serious harm to self or another may need frequent monitoring for progression to planning or intention to act on their thoughts. Such monitoring can be safely conducted at an outpatient level of care if providers and supports are adequately available and the patient can reliably participate in the monitoring process.(7)

Danger to self for adult

- Danger to self for adult is present due to **1 or more** of the following(1)(2)(3):
 - Command auditory hallucinations contributing to risk for suicide or serious Harm to self
 - Patient has persistent Thoughts of suicide or serious Harm to self that cannot be adequately monitored or treated at lower level of care as indicated by **1 or more** of the following[A]:
 - Necessary behavioral health care (such as required provider or lower level of care) not available or insufficient
 - Patient support system inadequate
 - Patient characteristics, such as moderately severe to severely Impaired impulse control, judgment, or insight; moderate to moderately severe Mania (adult) or unreliability
 - Indication or report of significant physical or sexual risky behavior with Impaired impulse control, judgment, or insight that significantly endangers self

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Footnotes

- A. Patients with persistent thoughts of suicide, homicide, or serious harm to self or another may need frequent monitoring for progression to planning or intention to act on their thoughts. Such monitoring can be safely conducted at an outpatient level of care if providers and supports are adequately available and the patient can reliably participate in the monitoring process.(2)

Danger to self for child or adolescent

- Danger to self for child or adolescent is present due to **1 or more** of the following(1)(2)(3)(4)(5)(6):
 - Command auditory hallucinations contributing to risk for suicide or serious Harm to self(4)
 - Patient has persistent Thoughts of suicide or serious Harm to self that cannot be adequately monitored or treated at lower level of care as indicated by **1 or more** of the following[A]:
 - Necessary child or adolescent behavioral health care (such as required provider or lower level of care) not available or insufficient
 - Conflict in family environment or other inadequacy in patient support system
 - Patient characteristics, such as moderately severe to severely Impaired impulse control, judgment, or insight; moderate to moderately severe Mania (child or adolescent) or unreliability
 - Indication or report of significant physical or sexual risky behavior with Impaired impulse control, judgment, or insight that significantly endangers self

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4. Sewall CJR, Goldstein TR, Brent DA. Youth suicide. In: Dulcan MK, editor. Dulcan's Textbook of Child and Adolescent Psychiatry. 3rd ed. American Psychiatric Association Publishing; 2022:551-564.
5. Shain B, Committee on Adolescence. Suicide and suicide attempts in adolescents. Pediatrics 2016;138(1):e20161420. DOI: 10.1542/peds.2016-1420. (Reaffirmed 2023 Jul)
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Footnotes

- A. Patients with persistent thoughts of suicide, homicide, or serious harm to self or another may need frequent monitoring for progression to planning or intention to act on their thoughts. Such monitoring can be safely conducted at an outpatient level of care if providers and supports are adequately available and the patient can reliably participate in the monitoring process.(4)

Discharge planning

- Discharge planning includes **ALL** of the following(1)(2)(3)(4)(5):
 - Identification of needed internal and external resources and services (eg, medical/care providers, care manager, housing, transportation, financial support)
 - Patient, family, guardian, and other supports involved in plan development as appropriate
 - Communication of current information to next level of care providers
 - Identification of contacts and their responsibilities at next level of care

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Evaluation completed and reviewed

- Completion of appropriate evaluation includes **ALL** of the following(1)(2)(3):
 - Psychiatric history and evaluation
 - Mental status examination
 - Evaluation of precipitants
 - Observation and symptom assessment completed or not needed
 - Physical examination completed or not indicated for level of care

References

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3. Shain B, Committee on Adolescence. Suicide and suicide attempts in adolescents. Pediatrics 2016;138(1):e20161420. DOI: 10.1542/peds.2016-1420. (Reaffirmed 2023 Jul)

Functional status acceptable

- Functional status acceptable, as indicated by **1 or more** of the following:
 - No essential function[A] is significantly impaired.
 - An essential function is impaired, but impairment is manageable/treatable at available lower level of care.

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1. American Association of Community Psychiatrists. Level of Care Utilization System for Psychiatric and Addiction Services (LOCUS). Adult version 20 [Internet] American Association of Community Psychiatrists. 2016 Dec Accessed at: <https://www.communitypsychiatry.org/keystone-programs/locus>. [accessed 2023 Sep 11]
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4. Child & Adolescent Service Intensity Instrument (CASII) User's Manual version 4.0. American Academy of Child and Adolescent Psychiatry 2014 Oct.

Footnotes

- A. Essential functions are those that are necessary to sustain life, such as feeding and hydrating oneself.(1)(2)(3)(4)

Harm

- Harm to self or another is considered serious if it has a substantial likelihood of causing death, disability, or major disfigurement.(1)(2)(3)(4)(5)

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1. American Association of Community Psychiatrists. Level of Care Utilization System for Psychiatric and Addiction Services (LOCUS). Adult version 20 [Internet] American Association of Community Psychiatrists. 2016 Dec Accessed at: <https://www.communitypsychiatry.org/keystone-programs/locus>. [accessed 2023 Sep 11]
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Impaired impulse control, judgment, or insight

- Impaired impulse control describes a lack of control or ability to regulate actions related to inner urges. Impaired judgment and insight may include a lack of recognition of current or past symptoms, denial of need for treatment, or lack of recognition of consequences of actions. Levels of severity may be described as (1)(2)(3):
 - Minimal (subtle symptoms that may rarely bother the patient and are not associated with interference with daily functioning (may represent extreme end of normal range))
 - Mild (symptoms may not be very bothersome to the patient, are only occasionally present, and contribute to **Mild** dysfunction in daily living for adult or **Mild** dysfunction in daily living for child or adolescent)
 - Mild to moderate (symptoms may not be very bothersome to the patient, are established (eg, present up to half of each week), and contribute to **Mild to moderate** dysfunction in daily living for adult or **Mild to moderate** dysfunction in daily living for child or adolescent)
 - Moderate (symptoms may be somewhat bothersome to the patient, are clearly established (eg, present more than half of each week, but not daily or near daily), and contribute to **Moderate** dysfunction in daily living for adult or **Moderate** dysfunction in daily living for child or adolescent)
 - Moderately severe (symptoms may be bothersome to the patient, are present more than half of each week (but not daily or near daily), and contribute to **Serious** dysfunction in daily living for adult or **Serious** dysfunction in daily living for child or adolescent but are not all-consuming and usually can be contained at will)
 - Severe (symptoms may be very bothersome to the patient, are present daily (or near daily), and contribute to **Severe** dysfunction in daily living for adult or **Severe** dysfunction in daily living for child or adolescent (eg, symptoms may be all-consuming or cannot be contained at will, and often require close supervision))
 - Extreme (symptoms may be highly disturbing to the patient, are present on an almost constant basis, and contribute to **Severe** dysfunction in daily living for adult or **Severe** dysfunction in daily living for child or adolescent (condition usually requires close supervision))

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3. Disruptive, impulse-control, and conduct disorders. In: American Psychiatric Association, editor. *Diagnostic and Statistical Manual of Mental Disorders*. DSM-5-TR ed. American Psychiatric Association; 2022:521-541.

Mania (adult)

- Mania symptoms may include elated/irritable mood, grandiosity, decreased need for sleep (eg, feeling rested after only a few hours of sleep), hypervolubal or rapid pressured speech, flight of ideas, racing thoughts, distractibility (ie, attention too easily drawn to unimportant or irrelevant external stimuli), increase in goal-directed activity (either socially, at work, or at school), hypersexuality, excessive energy, psychomotor agitation, or excessive involvement in pleasurable activities that have a high potential for negative consequences (eg, engaging in unrestrained buying sprees, sexual indiscretions, or foolish business investments). Symptoms do not have an alternative explanation (eg, intoxication, delirium, or other medical condition). Symptom severity may be described as (1)(2)(3)(4)(5)(6):
 - Mild (periods of somewhat elevated, expansive, or irritable mood, or other manic symptoms that are occasionally present)
 - Mild to moderate (periods of somewhat elevated, expansive, or irritable mood, or other manic symptoms that are present up to half the days of each week)
 - Moderate (periods of extensively elevated, expansive, or irritable mood or restlessness, or other manic symptoms that are present more than half the days of each week)
 - Moderately severe (frequent, but not daily, periods of extensively elevated, expansive, or irritable mood or restlessness, or other manic symptoms)
 - Severe (daily periods of extensively elevated, expansive, or irritable mood or restlessness, or other manic symptoms)

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Mania (child or adolescent)

- Mania symptoms may include elated/irritable mood, grandiosity, decreased need for sleep (eg, feeling rested after only a few hours of sleep), hypervoluble or rapid pressured speech, flight of ideas, racing thoughts, distractibility (ie, attention too easily drawn to unimportant or irrelevant external stimuli), increase in goal-directed activity (eg, socially or at school), hypersexuality, excessive energy, psychomotor agitation, or excessive involvement in pleasurable activities that have a high potential for negative consequences. Symptoms do not have an alternative explanation (eg, intoxication, delirium, or other medical condition).(1)(2)

References

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2. West AE, Celio CI, Henry DB, Pavuluri MN. Child Mania Rating Scale-Parent Version: a valid measure of symptom change due to pharmacotherapy. Journal of Affective Disorders 2011;128(1-2):112-9. DOI: 10.1016/j.jad.2010.06.013.

Medical needs absent or manageable/treatable at available lower level of care

- Medical needs absent or manageable/treatable at available lower level of care, as indicated by **ALL** of the following(1)(2)(3)(4)(5)(6):
 - Adverse medication effects absent or manageable/treatable
 - Medical comorbidity absent or manageable/treatable
 - Medical complications absent or manageable/treatable (eg, complications of eating disorder)
 - Substance-related disorder absent or manageable/treatable

References

1. American Association of Community Psychiatrists. Level of Care Utilization System for Psychiatric and Addiction Services (LOCUS). Adult version 20 [Internet] American Association of Community Psychiatrists. 2016 Dec Accessed at: <https://www.communitypsychiatry.org/keystone-programs/locus>. [accessed 2023 Sep 11]
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Mild dysfunction in daily living for adult

- **Mild** dysfunction in daily living for adult is present, as indicated by **1 or more** of the following(1)(2)(3)(4)(5)(6):
 - Some deterioration in interpersonal interactions (eg, increased conflict), but able to maintain some meaningful relationships
 - Some recent minor disruptions in self-care
 - Minor but consistent disruptions in social role functioning and meeting obligations, but still able to maintain these roles
 - Self-neglect with at least some minor limitation in ability to provide for basic needs (by self or others) in maintenance care
 - Other evidence of mild dysfunction

References

1. American Association of Community Psychiatrists. Level of Care Utilization System for Psychiatric and Addiction Services (LOCUS). Adult version 20 [Internet] American Association of Community Psychiatrists. 2016 Dec Accessed at: <https://www.communitypsychiatry.org/keystone-programs/locus>. [accessed 2023 Sep 11]
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3. WHO Disability Assessment Schedule 2.0 (WHODAS 2.0). 36-Item Instrument Scoring Sheet, Simple Scoring Calculation [Internet] World Health Organization. 2014 Nov Accessed at: <https://www.who.int/>. [accessed 2023 Aug 14]

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5. Sheehan DV, Harnett-Sheehan K, Raj BA. The measurement of disability. International Clinical Psychopharmacology 1996;11 Suppl 3:89-95.
6. Sheehan Disability Scale (SDS). [Internet] Psychiatry & Behavioral Health Learning Network. 2019 Jan 28 Accessed at: <https://www.psychcongress.com/saundras-corner/scales-screeners/disability-scales/sheehan-disability-scale-sds>. [created 1983; accessed 2023 Aug 11]

Mild dysfunction in daily living for child or adolescent

- **Mild** dysfunction in daily living for child or adolescent is present, as indicated by **1 or more** of the following(1)(2)(3)(4)(5)(6)(7):
 - Minor deterioration or episodic failure in relationships with peers, family, or adults (eg, defiance, lying, provocative behavior)
 - Self-care sporadically below usual or expected standards
 - Impairments in function characterized by ability to show improvements following episodes of deterioration
 - Diminished ability to assess consequences of own actions (eg, acts of severe property damage) that therapeutic intervention can remediate
 - Self-neglect with at least some minor limitation in ability to provide for basic needs (by self or others) in maintenance care
 - Other evidence of mild dysfunction

References

1. American Association of Community Psychiatrists. CALOCUS/CASII: Child and Adolescent Level of Care Utilization System. Child and Adolescent Version 20 [Internet] American Association of Community Psychiatrists. 2019 Jul Accessed at: <https://www.communitypsychiatry.org/keystone-programs/locus>. [accessed 2023 Sep 08]
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Mild to moderate dysfunction in daily living for adult

- **Mild to moderate** dysfunction in daily living for adult is present, as indicated by **1 or more** of the following(1)(2)(3)(4)(5)(6):
 - Some recent deterioration in interpersonal interactions (eg, conflicted, withdrawn), but maintains control of impulsive or abusive behaviors and maintains some meaningful relationships
 - Some recent disruptions in self-care below usual or expected standards
 - Disturbance in vegetative status (eg, eating, sleeping habits) that does not pose serious threat to health
 - Some deterioration in social role functioning and meeting obligations (eg, avoiding responsibilities on some occasions), but still can maintain roles overall
 - Other evidence of mild to moderate dysfunction

References

1. American Association of Community Psychiatrists. Level of Care Utilization System for Psychiatric and Addiction Services (LOCUS). Adult version 20 [Internet] American Association of Community Psychiatrists. 2016 Dec Accessed at: <https://www.communitypsychiatry.org/keystone-programs/locus>. [accessed 2023 Sep 11]
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4. WHO Disability Assessment Schedule 2.0 (WHODAS 2.0). 12-Item Instrument Scoring Sheet [Internet] World Health Organization. 2014 Nov Accessed at: <https://www.who.int/>. [accessed 2023 Aug 14]
5. Sheehan DV, Harnett-Sheehan K, Raj BA. The measurement of disability. International Clinical Psychopharmacology 1996;11 Suppl 3:89-95.
6. Sheehan Disability Scale (SDS). [Internet] Psychiatry & Behavioral Health Learning Network. 2019 Jan 28 Accessed at: <https://www.psychcongress.com/saundras-corner/scales-screeners/disability-scales/sheehan-disability-scale-sds>. [created 1983; accessed 2023 Aug 11]

Mild to moderate dysfunction in daily living for child or adolescent

- **Mild to moderate** dysfunction in daily living for child or adolescent is present, as indicated by **1 or more** of the following(1)(2)(3)(4)(5)(6)(7):
 - Some difficulty in relationships with peers, family, or adults (eg, defiance, lying), but without physical aggression
 - Self-care sometimes below usual or expected standards
 - Disturbance in vegetative status (eg, eating, sleeping habits) that does not pose serious threat to health
 - Deteriorating school behavior such that disruption in school is threatened (eg, threat of expulsion)
 - Diminished ability to assess consequences of own actions (eg, acts of severe property damage) that therapeutic intervention can remediate
 - Other evidence of mild to moderate dysfunction

References

1. American Association of Community Psychiatrists. CALOCUS/CASII: Child and Adolescent Level of Care Utilization System. Child and Adolescent Version 20 [Internet] American Association of Community Psychiatrists. 2019 Jul Accessed at: <https://www.communitypsychiatry.org/keystone-programs/locus>. [accessed 2023 Sep 08]
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7. Early Childhood Service Intensity Instrument (ECSII) for Infants, Toddlers, and Preschool-aged Children Ages 0-5 version 1. American Academy of Child and Adolescent Psychiatry 2009 Sep.

Moderate dysfunction in daily living for adult

- **Moderate** dysfunction in daily living for adult is present, as indicated by **1 or more** of the following(1)(2)(3)(4)(5)(6):
 - Recent conflicted, withdrawn, or troubled relationships, but maintains control of impulsive, aggressive, or abusive behaviors
 - Self-care frequently below usual or expected standards
 - Significant disturbance in vegetative status (eg, eating, sleeping habits), but not posing serious threat to health
 - Significant deterioration in ability to fulfill responsibilities (eg, work, commitments to significant others), including neglect or avoidance of these responsibilities on some occasions
 - Ongoing and variably severe deficits in interpersonal relationships and ability to engage socially in constructive manner
 - Other evidence of moderate dysfunction

References

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Moderate dysfunction in daily living for child or adolescent

- **Moderate** dysfunction in daily living for child or adolescent is present, as indicated by **1 or more** of the following(1)(2)(3)(4)(5)(6)(7):
 - Troubled relationships with peers, family, or adults (eg, conflicted, withdrawn), but without physical aggression

- Self-care frequently below usual or expected standards
- Significant disturbance in vegetative status (eg, eating, sleeping habits), but not posing serious threat to health
- Deteriorated school behavior such that disruption in school has occurred (eg, suspension, expulsion), and child is at risk for placement in alternative school setting or repeating grade
- Diminished ability to assess consequences of own actions (eg, acts of severe property damage)
- Other evidence of moderate dysfunction

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Risk status acceptable

- Risk status acceptable, as indicated by **ALL** of the following(1)(2)(3)(4)(5)(6):
 - Danger to self or others manageable/treatable, as indicated by **1 or more** of the following:
 - Absence of Thoughts of suicide, homicide, or serious Harm to self or to another
 - Thoughts of suicide, homicide, or serious Harm to self or to another present but manageable/treatable at available lower level of care
 - Patient and supports understand follow-up treatment and crisis plan.
 - Provider and supports are sufficiently available at lower level of care.
 - Patient, as appropriate, can participate as needed in monitoring at available lower level of care.

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Serious dysfunction in daily living for adult

- **Serious** dysfunction in daily living for adult is present, as indicated by **1 or more** of the following(1)(2)(3)(4)(5)(6):
 - Serious deterioration in quality of interpersonal interactions (eg, impulsive or abusive behaviors)
 - Significant withdrawal and avoidance of almost all social interaction
 - Consistent failure to achieve self-care (eg, personal hygiene, appearance) to usual standards
 - Serious disturbance in vegetative status (eg, weight change, sleep disruption) threatening physical function
 - Inability to perform close to usual standards in adult obligations (eg, work, parenting) and neglecting these responsibilities frequently or over extended period of time
 - Other evidence of serious dysfunction

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Serious dysfunction in daily living for child or adolescent

- **Serious** dysfunction in daily living for child or adolescent is present, as indicated by **1 or more** of the following(1)(2)(3)(4)(5)(6)(7):
 - Serious deterioration in interpersonal interactions (eg, impulsive or abusive behaviors)
 - Significant withdrawal and avoidance of almost all social interaction
 - Consistent failure to achieve self-care as appropriate to age or developmental level
 - Serious disturbance in vegetative status (eg, weight change, sleep disruption) threatening physical function
 - Inability to perform adequately in school (including specialized setting) due to disruptive or aggressive behavior
 - Severely diminished ability to assess consequences of own actions (eg, acts of severe property damage)
 - Other evidence of serious dysfunction

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Severe dysfunction in daily living for adult

- **Severe** dysfunction in daily living for adult is present, as indicated by **1 or more** of the following(1)(2)(3)(4)(5)(6):
 - Incapacitation because of grave disability (eg, severe regression with inability to provide for self)
 - Extreme deterioration in social interactions (eg, threatening behaviors with little or no provocation)
 - Complete withdrawal from all social interactions
 - Complete neglect of self-care with associated impairment in physical status
 - Extreme disruption in vegetative function (eg, life-sustaining functions such as eating)
 - Complete inability to maintain any appropriate aspect of personal responsibility in any adult roles (eg, occupational, parental)
 - Other evidence of severe dysfunction

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Severe dysfunction in daily living for child or adolescent

- **Severe** dysfunction in daily living for child or adolescent is present, as indicated by **1 or more** of the following(1)(2)(3)(4)(5)(6)(7):
 - Incapacitation because of grave disability (eg, severe regression with inability to provide for self)
 - Extreme deterioration in interactions with peers, adults, or family (eg, assaultive behaviors with no provocation, high levels of family conflict)
 - Complete withdrawal from all social interactions
 - Complete neglect of self-care with associated impairment in physical status
 - Extreme disruption in vegetative function (eg, life-sustaining functions such as eating)
 - Nearly complete inability to maintain any appropriate school behavior or academic achievement
 - Severely diminished ability to assess consequences of own actions (eg, acts of severe property damage)
 - Other evidence of severe dysfunction

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Social Determinants of Health Assessment

- Risk of poor health outcomes may be increased by the presence of **1 or more** of the following social determinants of health(1)(2)(3)(4):
 - Housing insecurity, as indicated by **1 or more** of the following:
 - Individual or caregiver's current living situation is **1 or more** of the following(5):
 - Does not have own housing (eg, staying in a hotel, shelter, or with others)
 - Has own housing (eg, house, apartment), but at risk of losing it in the future (ie, behind on rent or mortgage)
 - Has own housing (eg, house, apartment), but has lived in 3 or more places in past year
 - Current housing has **1 or more** of the following:
 - Electrical appliances (eg, stove, refrigerator) not working or unavailable
 - Insufficient heating or cooling
 - Insufficient ventilation
 - Lead paint or pipes
 - Mold
 - Pests (eg, bugs) or rodents
 - Smoke detectors not working or unavailable
 - Food insecurity, as indicated by **1 or more** of the following(6):
 - In the past year, individual or caregiver ran out of food and did not have money to buy more food.
 - In the past year, individual or caregiver worried that they would run out of food before they received money to buy more food.
 - Insufficient transportation, as indicated by **1 or more** of the following(7):

- In the past year, individual or caregiver missed medical appointments or could not get medications due to lack of transportation.
- In the past year, individual or caregiver missed nonmedical activities, work, or could not get things needed for daily living due to lack of transportation.
- Insufficient utilities, as indicated by **1 or more** of the following(8):
 - Utilities (eg, electricity, water, gas, or oil) are currently shut off or unavailable.
 - In the past year, electric, water, gas, or oil company threatened to shut off services.
- Personal safety risk, as indicated by **2 or more** of the following(6):
 - Individual is sometimes or frequently physically hurt by another person (including family member).
 - Individual is sometimes or frequently insulted or talked down to by another person (including family member).
 - Individual is sometimes or frequently threatened with physical harm by another person (including family member).
 - Individual is sometimes or frequently screamed or cursed at by another person (including family member).
- Insufficient dependent care, as indicated by **1 or more** of the following:
 - In the past year, individual or caregiver was unable to work due to lack of dependent care.
 - In the past year, individual or caregiver was unable to work more (additional) hours due to lack of dependent care.
 - In the past year, individual or caregiver missed medical appointments or could not get medications due to lack of dependent care.
 - In the past year, individual or caregiver missed nonmedical activities (eg, school, church, social activity) due to lack of dependent care.
- Depression risk, as indicated by **ALL** of the following:
 - In the past 2 weeks, individual had little interest or pleasure in normal activities on at least several days.
 - In the past 2 weeks, individual felt down, depressed, or hopeless on at least several days.

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Thoughts of suicide

- Thoughts of suicide or suicidal ideations are defined as "thoughts of serving as the agent of one's own death," to be distinguished from thoughts of death that do not involve actively bringing death about.(1)(2)

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